

1. NAME OF THE MEDICINAL PRODUCT

K TAB SR Tablet 750mg

2. ACTIVE INGREDIENTS AND THEIR QUANTITIES

One prolonged-release tablet contains 750mg potassium chloride (the tablet contains 10mmol of potassium)
For a complete list of excipients, see section 6.1

3. PHARMACEUTICAL FORM

Opaque white to pearl white, oval, plain tablet

4. CLINICAL DATA

4.1 Indications for use

Hypokalemia.

Prevention of hypokalemia in connection with diuretic therapy.

4.2 Dosage and method of administration

Dosage

Adults:

Prophylactic use.

750mg: Usually 1-2 tablets 2-3 times a day.

In hypokalemia, the dose is determined individually based on the serum potassium concentration. Two 750mg tablets 2-3 times a day are usually sufficient until serum potassium levels have been corrected. After that, the dose is usually 1-2 tablets twice a day.

Serum potassium should be determined regularly so that the dosage can be adjusted depending on the effect.

Elderly patients:

For the elderly with normal kidney function, the same dosages as for adults with normal kidney function can be used. Since some elderly people may have impaired kidney function, the dose may need to be changed according to the state of kidney function (see section Renal impairment).

Paediatric patients:

The safety and efficacy of this product in the treatment of children under 18 years of age have not been confirmed. Information is not available.

Kidney deficiency:

Individual dosage reduction is necessary for patients with mild or moderate renal impairment. This product must not be used in patients with severe renal impairment (see sections 4.3 and 4.4).

Method of administration

The tablets must be swallowed whole with at least one glass of water in a position other than lying down.

4.3 Contraindications

- Hypersensitivity to excipients mentioned in Section 6.1
- Hyperkalemia and conditions that may lead to hyperkalemia (see sections 4.4 and 4.5)
- Severe renal impairment (see section 4.4)
- Ulcers and strictures of the gastrointestinal tract (see section 4.4)
- Untreated Addison's disease (see section 4.4)

4.4 Warnings and precautions related to use

Potassium chloride should be dosed with great caution in patients with heart disease or a condition predisposing to hyperkalemia, such as renal or adrenal insufficiency, acute dehydration, or extensive tissue destruction (e.g. in severe burns). Serum potassium should be monitored in patients with cardiac or renal failure. Potassium chloride should be dosed with caution in the elderly, as they have an increased risk of heart or kidney failure. Strictures, bleeding, ulcers and perforations of the upper and lower gastrointestinal tract may occur, especially if too little water is taken with the dose of potassium chloride or when the patient has slowed intestinal transit.

For this reason, potassium chloride should be administered with caution to patients with slowed intestinal transit, such as bedridden patients or pregnant women (see section 4.6). The treatment must be stopped if severe nausea, vomiting or stomach problems occur.

4.5 Interactions with other medicinal products and other interactions

Concomitant use of potassium-sparing diuretics (spironolactone, eplerenone, triamterene and amiloride), angiotensin converting enzyme (ACE) inhibitors (e.g. captopril, enalapril, lisinopril, perindopril, quinapril, ramipril, zofenopril), angiotensin II receptor blockers (e.g. acilsartan, candesartan, eprosartan, irbesartan, losartan, tasosartan, telmisartan, valsartan), renin inhibitors (e.g. aliskiren), cyclosporine, tacrolimus, trimethoprim and potassium-containing drugs such as potassium salts of penicillin increase the risk of hyperkalemia.

4.6 Fertility, pregnancy and breastfeeding

Pregnancy:

There are no or limited data from the use of potassium chloride in pregnant women. Potassium chloride should be administered with caution to patients with delayed intestinal transit, such as pregnant women (see section 4.4). Potassium chloride should only be used during pregnancy if the potential benefits outweigh the potential risks.

Nursing:

Potassium is excreted in human breast milk, but when using normal treatment doses of potassium chloride prolonged-release tablets, no effects on newborns/infants are expected. This product can be used during breastfeeding.

Fertility:

The effect of potassium chloride on fertility has not been studied.

4.7 Effect on the ability to drive and use machines

Potassium chloride has no harmful effect or has a minor effect on the ability to drive and use machines.

4.8 Adverse effects

Adverse effects have been determined from literature sources and spontaneous reports. Gastrointestinal disturbances may occur. Hyperkalemia and gastrointestinal ulceration or bleeding have been reported.

Adverse effects are presented in each MedDRA organ class in descending order according to the severity of the adverse effect. The frequency of side effects cannot be estimated due to lack of data.

Metabolism and nutrition	
	Hyperkalemia
Digestive system	
	Gastrointestinal perforation Gastrointestinal bleeding Peptic ulcer

	Duodenal ulcer Esophageal ulcer Gastrointestinal stenosis Collapse of the gastrointestinal tract Diarrhea Vomit Stomach ache Nausea
Skin and subcutaneous tissue	
	Rash*

*Different types of rash reactions such as erythematous, maculopapular, popular, pruritic, and pustular rash have been described.

4.9 Overdose

Excessive doses of potassium lead to the development of hyperkalemia, especially in patients with renal failure. Overdose symptoms include confusion, paresthesia of the extremities, muscle weakness, paralysis, hypotension, cardiac arrhythmias, cardiac conduction block, and cardiac arrest.

ECG changes are an important sign of potassium poisoning.

Treatment according to local guideline. The following may be considered: gastric emptying if necessary. ECG monitoring must be started. Administration of insulin and glucose followed by oral or rectal polystyrene sulfonate. Sodium bicarbonate by intravenous infusion or calcium chloride or calcium gluconate by intravenous injection.

Toxic dose: 169 mmol.

In smaller children, cardiac arrest can occur after 60 mmol.

Severe symptoms are observed after a dose of approximately 3mmol/kg, and death at 4-13 mml/kg after taken the dose orally. Because there is no clear correlation between severity of hyperkalemia and the likelihood of life-threatening arrhythmias, individualized treatment must take into account underlying conditions such as renal function and concomitant medications that predispose to abnormal intracellular or extracellular potassium balance and abnormal potassium excretion.

5. PHARMACOLOGICAL PROPERTIES

5.1 Pharmacodynamics

Pharmacotherapeutic group: Minerals, ATC code: A12BA01

Potassium chloride is released from the tablet gradually in the small intestine, reducing the risk of side effects such as unspecified ulcers. The core of tablet is formed by a white and soft lipid core, which is eliminated with the feces. The film coating of the tablets makes it easier to swallow and masks the bitter taste.

Potassium ion is an important cation involved in key physiological processes such as maintaining intracellular tonicity, transmission of nerve impulse, muscle contraction and maintenance of normal kidney function.

Potassium is normally obtained with food, and when the body is in a state of balance, the same amount of potassium is absorbed from the gastrointestinal tract as is removed from the body with urine. The normal daily intake is 50-100 mEq per day. Decreased intake or increased excretion of potassium leads to symptoms of hypokalemia.

P5.2 Pharmacokinetics

Potassium is easily absorbed from the gastrointestinal tract and moves into the intracellular fluid so that the potassium content is about 150 mEq/l. The normal plasma potassium concentration is estimated to be 3.5-5 mEq/l.

Potassium is mainly excreted through the renal tubules.

The structure of the prolonged-release tablets is such that potassium chloride is slowly released in the gastrointestinal tract over 6-8 hours.

P5.3 Preclinical safety data

Not applicable

6. PHARMACEUTICAL INFORMATION

6.1 Excipients

Silica colloidal, Ethyl cellulose, Steryl alcohol, Surelease Clear, Hypromellose, Talc

6.2 Incompatibilities

Not applicable

6.3 Shelf life

36 months

6.4 Storage condition

Store below 30°C.

6.5 Nature and contents of container

Available in blister packs of 10 tablets. Box of 10x10 blisters.

7. Product Registration Holder

Mansa Healthcare Sdn. Bhd.
B-05-3A, 3 Two Square, No 2, Jalan 19/1, 46300 Petaling Jaya, Selangor

8. MANUFACTURER

Centaur Pharmaceuticals Private Limited
Plot No. 4, International Biotech Park, Hinjewadi, Phase II, Pune, Maharashtra 411057, India

9. DATE OF REVISION

1-Aug-2025

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